جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : فردوس محمد محمد البلاط
المحافظة : القليوبيه **الحي / المدينة :** اجهور الكبرى **العنوان :** ى
رقم البطاقة : 26403011401487 **رقم التليفون :** 01009592358 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 01-03-1964 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي الاورام المنتشر [14456] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فردوس محمد محمد البلاط
National ID: 26403011401487



تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

مركز دلتا للأشعة
Delta SCAN

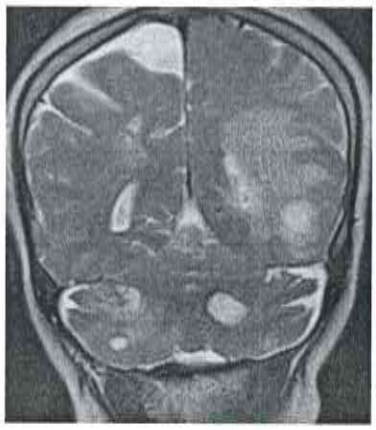
Study Date: 2026/09/04 05:45:20 PM	اسم المريض: فردوس محمد البلاط
الطبيب المعالج: اسماء اسماعيل	كود المريض: 376704

POST-CONTRAST MRI STUDY OF THE BRAIN

❖ TECHNIQUE:-

- *Plate 1-3: Axial T1, T2WIs & FLAIR*
- *Plate 4: Coronal T2 and sagittal T1WI.*
- *Plate 5: DWIs and ADC mapping.*
- *Post-contrast series.*

❖ FINDINGS:



- **Bilateral cerebral cortical and subcortical gray/ white matter interface multiple well-defined intra-axial cystic S.O.Ls are seen, the largest seen at the left temporal region measuring about 4.3x2.8x3cm cm along its maximum axial and CC dimensions respectively, elicits intermediate to low T1 / FLAIR and heterogenous high T2 signals showing marginal post-contrast enhancement.**
- **These lesions are surrounded by moderate vasogenic edema; together exert mass effect in the form of effacement of the adjacent cortical sulci and compression of related parts of both lateral ventricles and mild midline shift to the right about 4mm.**
- **Other similar but smaller SOLs are seen at bilateral cerebellar hemispheres.**
- No brain stem areas of abnormal MRI signal intensity.
- No intra or extra axial collections.

❖ OPINION:

- ***Bilateral cerebral and cerebellar well-defined intra-axial S.O.Ls with vasogenic edema causing mass effect as described, metastatic deposits is first to be considered rather than inflammatory with abscesses formation, for clinical correlation and MR spectroscopic assessment.***

Much Obliged, Yours
Dr. Ahmad Nassar, MD

فرع شبرا الخيمة
شارع ١٥ مايو - محطة المرور - بجوار البنك الأهلى وبنك قطر الوطنى
01281130055 02-42222279 01030711113
فرع مسطرد: ٤ ش ١٥ مايو - مسطرد (شبرا الخيمة ثانى)
02-44731775 01287000034

فرع شبين القناطر
وحدة الرنين المغناطيسى برج الياسمين موقف مسطرد
وحدة الأشعة المقطعية - شارع بورسعيد بجوار ستوديو ميامى
0132711199 01273339932 - 01200043245



ERM036723173

٤٢٨٩

٠١٠٠٩٥٩٤٢٥٨

National ID - Front
[[DOCTYPE:ID_FRONT]]

Name: فردوس محمد محمد البلاط **MR. :** 13819	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has a history of bilateral breast cancer diagnosed in 2024, for which neoadjuvant chemotherapy with the TCHP regimen was administered. Follow-up was missed after May 2024 and no surgical intervention was performed. On the current presentation, brain MRI demonstrated bilateral cerebellar and cerebral well-defined intra-axial lesions measuring 4.3 × 2.8 × 3 cm, associated with vasogenic edema and mass effect, consistent with metastatic deposits. The patient is receiving dehydration measures. Physical examination revealed a right-sided breast mass that is locally advanced, with a positive range and retracted nipple, and a left-sided breast mass measuring approximately 3 × 2 cm; no axillary lymphadenopathy was noted. The plan includes a new work-up with contrast-enhanced CT of the chest, abdomen, and pelvis, as well as bilateral breast ultrasound.

تحريرا في ,

التاريخ :	20-09-2026	**الوقت :**	

الطبيب المعالج : nada mohamed elsayed

يعتمد ,



National ID – Back
[[DOCTYPE:ID_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : فردوس محمد محمد البلاط Sex: Female
3- Medical number: 83174020
Old Med : 4389/2023
2-ID number:
Patient Age : 60 Y

Family history	Family History -VE	Phone	Phone 01009592358	BMI	BMI 32	REASON OF MDT	REASON OF MDT REMDT
marital status	Marital Status	NRS Score	NRS Score 1	GROUP	GROUP B	Date of MDT	Date Of MDT 31/08/2024
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
RT breast pathologically proven diffuse malignant process , Ipsilateral LNS
LT Breast UOQ Bergin lesions (BIRAD 3)

Mets Workup :
CAP : Bilad minimal pleural effusion enlarged LT thyroid lobe
Bone scan : free

BX	ER	PR	Her 2 Neu	SISH	Ki67
BX COMMENT: RT breast : IDC LT breast : Invasive lobular carcinoma	ER COMMENT: RT 0/8 LT 8/8	PR COMMENT: RT 0/8 LT 7/8	Her 2 Neu COMMENT: RT +VE LT -VE	SISH COMMENT	

T	N	M
RT 4B LT 1	RT 1 LT 0	0

Sonomammography

PATIENT HISTORY
PT RECIEVED 6 CYCLES TCHP

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
FOR LT BCS + SLNB /
FOR RT MASTECTOMY +/_ SSM

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ منير القراوي	RADIOLOGY	RADIOLOGY د/ محمد محسن
SURGERY	SURGERY د/ [illegible]	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition شراء حربي

31-8-24

Patient Name	: الأستاذه / فردوس محمد محمد البلاط	Request No.	: 13005357
Patient No.	: 26403011401487	Request Date	: 22/11/2023
Path No.	: SR -1444/23	Verification Date	: 05/12/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 59 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
Right& left breast massess

Nature of specimen
Revision of slides
Gross examination
Two blocks (7753/23) &6 slides were received .

Microscopic examination
1)Breast mass
* Histological type: Invasive duct carcinoma with calcification
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
* DCIS: Focal
* Angioinvasion: Could not be detected within the limits of biopsy received
IPT :
ER:No nuclear staining in tumor cells(0/8)
PR:No nuclear staining in tumor cells (0/8)
Her2: Strong complete memrane staining in >10 of tumor cells , score(+3)
2)Breast mass:
* Histological type: Invasive lobular carcinoma
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
* DCIS: No
* Angioinvasion: Could not be detected within the limits of biopsy received
IPT :
ER:Strong nuclear staining in 95% of tumor cells(8/8)
PR:Strong nuclear staining in 60% of tumor cells (7/8)
Her2: No memrane staining in tumor cells , score(0)
Diagnosis

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name: فردوس محمد البلاط

Report Date: 23/12/2023

Ref. Phy.:

Echo Doppler study

Measurements:

AO root	3.0cm	LVEDD	5.3cm
LA	3.9cm	LVESD	3.5cm
IVS	0.9cm	EF%	63%
LVPW	0.9cm	RV	1.2cm

Comment:

- Normal LV internal dimensions with good LV systolic function.
- Left ventricular diastolic dysfunction (grade II).
- No RWMA can be detected at rest.
- Normal mitral valve structure.
- Normal aortic valve structure and flow signal
- Normal LA dimension.
- Normal RV dimension with preserved systolic function (TAPSE=23mm) with mild TR of normal valve with normal PASP.
- Normal IVC diameter and collapse.
- Intact IAS and IVS by TTE.
- Normal pericardium.
- No intra_cavitary thrombi or masses by TTE.

Conclusion:

❖ **Left ventricular diastolic dysfunction.**

Best Regards,

Dr. Shaimaa Mostafa (MD)

Professor of cardiovascular medicine

European board of Echocardiography

PATIENT NAME: FARDOS MOHAMMED MOHAMMED EL BLAT
GENDER: Female
DATE : 11-Nov-2023 12:22 PM

PATIENT ID: 20223003440
AGE: 59 y
ORDER: 2566639

Report:

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy Elsayed **12-Nov-2023 04:01 PM**

www.Bahoya.org 16602
مركز بهية الهرم
4 شارع علوية أول شارع الهرم بجوار داري . الجيزة
مستشفى بهية الشيخ زايد
١٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Fardos Mohamed Mohamed Alballat	Date : 11/08/2024
Exam : MRI RIGHT KNEE	Age : 60 Year(s)
Medical ID: 4389/2023	Pat. ID: 48698

MRI OF THE RIGHT KNEE JOINT

Technique of examination:

- Sagittal T1, PD & T2 WIs
- Axial T2 Wis.
- Coronal STIR WIs.

MRI Findings:

- Maintained tibiofemoral and patellofemoral joints articular relations
- Osteoarthritic knee changes are seen with narrowed tibiofemoral joint space more at lateral aspect with subchondral degenerative changes of opposing articular surfaces, marginal tibial and femoral condylar osteophytic lippings with bipolar patellar lippings
- Triangular intermediate signal is seen within the posterior horn of the medial meniscus not reaching its articular surfaces.
- Normal configuration and MR signal of the lateral meniscus
- Elevated signal of ACL fibers with intact fibers continuity
- Normal MRI appearance of the PCL, collateral ligaments, quadriceps tendon and infrapatellar ligament.
- Mid knee joint effusion is noted.
- Large septated multi-locular baker cyst seen with minimal fluid signal caudal to it.
- Intact peri-articular musculature with preserved intervening fat planes.

Opinion:

- Osteoarthritic knee changes
- Degenerated posterior horn medial meniscus.
- ACL sprain
- Mild knee joint effusion with picture suggestive of leaking Baker's cyst

Signature Dr. Ibrahim Abo El Magd

18/08/2024 01:29 م

M.R.I
[[DOCTYPE:MRI]]

Name: Ferdous Mohammed Mohammed

Date: 18/12/2023

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: A case of right invasive duct carcinoma and left invasive lobular carcinoma

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: minimal background enhancement

ACR: b (scattered fibro-glandular densities)

Right BREAST:

- **Low energy (Non contrast) images**: Diffuse pathological process seen inform of asymmetrical density , distortion with overlying malignant microcalcifications , the process is more evident in the upper quadrants and centrally .
- Diffuse skin thickness
- **Recombined (Post contrast) images**: Diffuse heterogeneous moderate non mass enhancement seen involve most of the breast parenchyma , more evident in the upper quadrants , it is seen located 1.2 cm from the nipple and measures 13x12.7x10cm . **(this corresponding to the calcifications coverage area).**
- **Complementary US**:
- Pathologically enlarged level I lymph nodes, (more than 3 in number) all are discrete , globular in shape with lost hilum, the largest is25mm in diameter.

Left BREAST:

- **Low energy (Non contrast) images**
  1. UIQ deeply seated area of distortion.
  2. UOQ two circumscribed middle and anterior third isodense oval lesions, the largest shows few calcific foci within
  3. Area of grouped punctated microcalcifications bout 6mm in diameter located at about 6.5cm from the nipple
- **Recombined (Post contrast) images**
- UIQ deeply seated faint enhancing lesion measures 21x20x19mm located 13cm from the nipple.
- **Complementary ultrasound:**
  1. UOQ 3 o'clock two **non enhancing** well circumscribed isoechoic mass lesions, they measure 9x5mm located at about 7cm from the nipple and 10mm from the skin and the second one measures 6x5mm located 4cm from the nipple and 6mm from the skin . **corresponding to the mammographic finding**
  2. 10-11 o'clock irregular hypoechoic mass with spiculated margins , **corresponding to mammographic and contrast finding**
  3. Solitary **indeterminate** level I axillary lymphnode show diffuse cortical thickness up to 2.5mm , however it is kidney shaped with preserved hilum

IMPRESSION:

RIGHT BREAST: Locally advanced malignant process with finding as described (BIRADS6 = pathologically proved malignancy I)

LEFT BREAST:

1-UIQ unifocal malignant enhancing lesion ((BIRADS6 = pathologically proved malignancy)

2-UOQ two non enhancing probably benign lesion (BIRADS3= probably benign)

3- UOQ grouped punctate calcifications(BIRADS3= probably benign)

Best regards,

Dr. Marwa Ramzy, FRCR

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Fardos Mohamed Mohamed Alballat	Date : 27/07/2024
Exam : MAMMOGRAPHY BOTH	Age : 60 Year(s)
Medical ID: 4389/2023	Pat. ID: 48698

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Bilateral breast cancer

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are Scattered fibro glandular density (ACR B)
- Right breast diffuse malignant looking microcalcifications involving most of the breast parenchyma, reaching to the nipple, with no underlying findings see by US
- Left UIQ spiculated mass of high density with radioopaque clip seen within measuring 2.2x2.5 cm, seen 8.2 cm from the nipple. US showed 10:00 spiculated hypoechoic solid mass measuring 1.6x1.2 cm seen 0.29 cm from the overlying skin and indenting the underlying muscle
- Normal skin thickness and nipple-areola complex.
- Bilateral multiple non-specific axillary LNs, more prominent in the left axilla

Opinion:

- Right multicenteric pathologically proved malignant microcalcifications with complete response of ipsilateral axillary lymph nodes (BIRADS 6)
- Left UIQ partial response of pathologically proved malignant mass (BIRADS 6)

Yours truly

Signature Dr. Yasmin Nada

27/07/2024 3:00 PM

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

Patient Name	: الأستاذه / فردوس محمد محمد البلاط	Request No.	: 13005357
Patient No.	: 26403011401487	Request Date	: 22/11/2023
Path No.	: SR -1444/23	Verification Date	: 05/12/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 59 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
Right& left breast massess

Nature of specimen
Revision of slides
Gross examination
Two blocks (7753/23) &6 slides were received .

Microscopic examination
1)Breast mass
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* DCIS: Focal
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ER:No nuclear staining in tumor cells(0/8)
PR:No nuclear staining in tumor cells (0/8)
Her2: Strong complete memrane staining in >10 of tumor cells , score(+3)
2)Breast mass:
* Histological type: Invasive lobular carcinoma
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
* DCIS: No
* Angioinvasion: Could not be detected within the limits of biopsy received
IPT :
ER:Strong nuclear staining in 95% of tumor cells(8/8)
PR:Strong nuclear staining in 60% of tumor cells (7/8)
Her2: No memrane staining in tumor cells , score(0)
Diagnosis

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

Patient Name	: الأستاذه / فردوس محمد محمد البلاط	Request No.	: 13005357
Patient No.	: 26403011401487	Request Date	: 22/11/2023
Path No.	: SR -1444/23	Verification Date	: 05/12/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 59 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

1) Breast mass ; slide revision :
Invasive duct carcinoma, Grade II with focal DCIS with calcification
IPT :
ER: Negative (score 0/8)
PR:Negative (score 0/8)
Her2: Positive score(+3)
2) Breast mass ; slide revision :
Invasive lobular carcinoma, Grade II
IPT :
ER: Positive (score 8/8)
PR:Positive (score 7/8)
Her2: Negativescore(0)

Signature
Dr. Rana Fathy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

[[DOCTYPE:ULTRASOUND]]
Ultrasound

Name: **fardoos Mohamed Mohamed**
Date: **26.2.204**

Left BREAST ULRASOUND GUIDED CLIP LOCALIZATION:

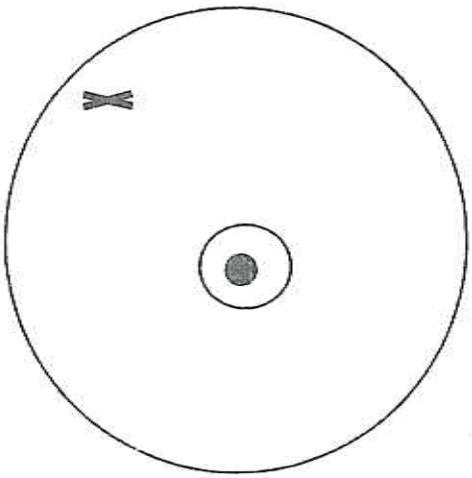
CLINICAL SUMMARY:

left breast pathologically proven CA, for one clip insertions in the UIQ mass lesion

TECHNIQUE:

Under complete sterile conditions, 2ml of local anaesthesia were administered at the sites of concern in the left breast.
one clip marker was inserted within the centre of the mass, at 11 o'clock about 12 cm away from the nipple.

Post procedure ML and CC views were done to confirm.



Best regards,
Dr. Marwa M.Hassan

ت / فـ: ٠٢٢٣٦٤٥٨١٣
Tel, Fax: 0223645813

١٠٥٣ ش كورنيش النيل . قم الخليج . القاهرة
1053 Corniche El Nil St. Fom Al Khaleeg . Cairo

[[DOCTYPE:MAMMOGRAM]]

PATIENT NAME: FARDOS MOHAMMED MOHAMMED EL BLAT
GENDER: Female
DATE : 06-Nov-2023 12:34 PM

PATIENT ID: 20223003440
AGE: 59 y
ORDER: 234979,234980

Report: - all studies

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right breast pathologically proven IDC.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).
- Right breast global asymmetry with overlying global microcalcifications extending to the nipple.
- Left breast inner rather central deeply seated focal asymmetry. Left UOQ circumscribed intermediate dense lesion also noted.
- This was confirmed by **Complementary Ultrasound** which showed
- Right breast diffuse pathological process in form of altered hypoechoic vascular parenchyma reaching to the nipple with subsequent nipple retraction, dilated subdermal lymphatics and overlying skin thickening.
- Left breast 10/ 11 O'clock irregular spiculated hypoechoic lesion surrounding by desmoplastic reaction and measuring 16 x 13 mm. it is away from the skin by 3.3 mm.
- Left breast UOQ two well defined hypoechoic oval lesions seen at 1 O'clock measuring 9 x 5 mm and at 2 O'clock measuring 7 x 5 mm.
- Normal left skin thickness and contour.
- Normal left nipple areola complexes.
- Right multiple enlarged axillary lymph nodes (level I,II) showing irregular cortical thickening and muffled hilum.
- Prominent right single supraclavicular lymph node.
- Left few axillary lymph nodes showing cortical thickening reaching 0.4 mm.

OPINION:

- Right breast pathologically proven diffuse malignant process as described along with ipsilateral infiltrated lymph nodes (level I.II) (BIRADS6).
- Left breast 10/11 O'clock highly suspicious lesion for histopathological

[[DOCTYPE:BONE_SCAN]]
Bone Scan

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

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Date : 12-10-2026 08:21

Branch : Dokki

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Tc-99m MDP WHOLE-BODY BONE SCINTIGRAPHY REPORT

Clinical Indication: Follow-up skeletal metastatic survey in a 62-year-old female with right breast carcinoma, presenting with active bone pain.

Procedure: Whole-body anterior and posterior planar bone scintigrams were acquired approximately 2 to 3 hours after I.V Tc-99m MDP administration.

Findings:

- Focal pathologically increased uptake is seen at the right frontoparietal bone. Multiple intense patchy focal areas of increased uptake involve numerous dorsolumbar vertebrae, with focal hyperactive retention in the sternum and a few bilateral ribs. Asymmetrical focal hyperfixation is scattered throughout the pelvic bones.
- Focal uptake is seen at the left shoulder girdle, with diffuse heterogeneous uptake and suspected early marrow infiltration along both humeral and femoral shafts.
- Symmetrical renal visualization with physiological bladder accumulation.

Impression:

- Widespread multifocal osteoblastic skeletal metastatic disease involving the right frontoparietal bone, numerous dorsolumbar vertebrae, sternum, bilateral ribs, pelvic bones, the left shoulder girdle, and suspected marrow infiltration along both humeral and femoral shafts.
- Radiological correlation and follow up as recommended as clinically indicated.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER